

FRONT



SALBUTAMOL CFC-Free Metered Dose Inhaler 100mcg / actuation

VISALxx-0_4

Product Description

10 ml white and homogenous suspension aerosol dosed for 200 inhalations (MDI).

Aluminium aerosol can with 200 inhalations (10ml), containing 100 µg of salbutamol each acutuation, sealed with a metering valve (50 µl of capacity), equipped with a polypropylene plastic oral actuator-adapter (or mouthpiece) and polypropylene plastic cap. These elements are the ones that are usually used in medicines for inhalatory use. The colour of the inhaler is blue actuator with dark blue cap.

Composition

Each actuation contains 120 mcg of Salbutamol Sulphate (equivalent to Salbutamol 100 mcg).

Pharmacodynamics

Salbutamol is a selective beta²-adrenoceptor agonist. At therapeutic doses it acts on the beta²-adrenoceptor of bronchial muscle providing short acting (4 to 6 hour) bronchodilation with a fast onset (within 5 minutes) in reversible airways obstruction.

Pharmacokinetics

Absorption

After administration by the inhaled route, between 10 and 20% of the dose reaches the lower airways. The remainder is retained in the delivery system or is deposited in the oropharynx from where it is swallowed. The fraction deposited in the airways is absorbed into the pulmonary tissues and circulation but is not metabolised by the lung.

Distribution

Salbutamol is bound to plasma proteins to the extent of 10%.

Metabolism

On reaching the systemic circulation, salbutamol becomes accessible to hepatic metabolism and is excreted, primarily in the urine, as unchanged drug and as the phenolic sulphate. The swallowed portion of an inhaled dose is absorbed from the gastrointestinal tract and undergoes considerable first-pass metabolism to the phenolic sulphate. Both unchanged drug and conjugate are excreted primarily in the urine.

Elimination

Salbutamol administered intravenously has a half-life of four to six hours and is cleared partly renally and partly by metabolism to the inactive 4'-O-sulphate (phenolic sulphate) which is also excreted primarily in the urine. The faeces are a minor route of excretion. The majority of a dose of salbutamol given intravenously, orally or by inhalation is excreted within 72 h.

Indication

CP Pharma Salbutamol CFC-Free Metered Dose Inhaler 100mcg/actuation provides short-acting (4 to 6 hour) bronchodilation with fast onset (within 5 minutes) in reversible airway obstruction.

It is particularly suitable for the relief and prevention of asthma symptoms. CP Pharma Salbutamol CFC-Free Metered Dose Inhaler 100mcg/actuation should be used to relieve symptoms when they occur, and to prevent them in those circumstances recognised by the patient to precipitate an asthma attack (e.g. before exercise or unavoidable allergen exposure).

CP Pharma Salbutamol CFC-Free Metered Dose Inhaler 100mcg/actuation is particularly valuable as relief medication in mild, moderate or severe asthma, provided that reliance on it does not delay the introduction and use of regular inhaled corticosteroid therapy.

Contraindications

CP Pharma Salbutamol CFC-Free Metered Dose Inhaler 100mcg/actuation is contraindicated in patients with a history of hypersensitivity to any of its components (see Excipients). Non-i.v. formulations of salbutamol must not be used to arrest uncomplicated premature labour or threatened abortion.

Warning and Precautions

The management of asthma should normally follow a stepwise programme, and patient response should be monitored clinically and by lung function tests.

Increasing use of short-acting bronchodilators, in particular beta-2 agonists to relieve symptoms indicates deterioration of asthma control. Under these conditions, the patient's therapy plan should be reassessed.

Sudden and progressive deterioration in asthma control is potentially life-threatening and consideration should be given to starting or increasing corticosteroid therapy. In patients considered at risk, daily peak flow monitoring may be instituted.

CP Pharma Salbutamol CFC-Free Metered Dose Inhaler 100mcg/actuation should be administered cautiously to patients with thyrotoxicosis.

Potentially serious hypokalaemia may result from beta-2 agonist therapy mainly from parenteral and nebulised administration.

Particular caution is advised in acute severe asthma as this effect may be potentiated by concomitant treatment with xanthine derivatives, steroids, diuretics and by hypoxia. It is recommended that serum potassium levels are monitored in such situations.

As with other inhalation therapy, paradoxical bronchospasm may occur, resulting in an immediate increase in wheezing after dosing. This should be treated immediately with an alternative presentation or a different fast-acting inhaled bronchodilator, if immediately available, CP Pharma Salbutamol CFC-Free Metered Dose Inhaler 100mcg/actuation should be discontinued, and if necessary a different fastacting bronchodilator instituted for ongoing use.

In the event of a previously effective dose of inhaled CP Pharma Salbutamol CFC-Free Metered Dose Inhaler 100mcg/actuation failing to give relief for at least three hours, the patient should be advised to seek medical advice in order that any necessary additional steps may be taken.

The patient's inhaler technique should be checked to make sure that aerosol actuation is synchronised with inspiration of breath for optimum delivery of the drug to the lungs.

Interactions with other medicaments

CP Pharma Salbutamol CFC-Free Metered Dose Inhaler 100mcg/actuation and non-selective beta-blocking drugs, such as propranolol, should not usually be prescribed together.

Side Effects

System Organ Class	Frequency	Adverse Event
Immune system disorders	Very rare	Hypersensitivity reactions (angioedema, urticaria, bronchospasm, hypotension and collapse)
Metabolism and nutrition disorders	Rare	Hypokalaemia ,increased serum lactate levels and acidosis lactic
Psychiatric disorders	Common	Tenseness
	Rare	Sleep disturbances and hallucinations (especially in children), hyperactivity in children
	Very rare	Insomnia
Nervous system disorders	Common	Tremor muscle, headache, dizziness
Cardiac disorders	Rare	Palpitations, tachycardia
	Very rare	Cardiac arrhythmia including atrial fibrillation, supraventricular tachycardia and extrasystoles – especially if used concomitantly with other β ₂ - agonists
	Not known	Myocardial ischaemia
Vascular disorders	Rare	Peripheral vasodilatation
Respiratory, thoracic and mediastinal disorders	Rare	Throat irritation
	Very rare	Paradoxical bronchospasm (with an immediate increase in wheezing after dosing)
Gastrointestinal disorders	Rare	Mouth irritation, nausea, vomiting, dry mouth, sore mouth
Skin and subcutaneous tissue disorders	Very rare	Pruritus
Musculoskeletal and connective tissue disorders	Uncommon	Myalgia
	Rare	Muscle cramps
	Very rare	Fine tremor (particularly of hands)

As with other inhalation therapies, paradoxical bronchospasm may occur immediately after dosing. CP Pharma Salbutamol CFC-Free Metered Dose Inhaler 100mcg/actuation should be discontinued immediately, the patient reassessed and treated immediately with another presentation or a different fast-acting inhaled bronchodilator.



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Pregnancy and Lactation

Fertility
There is no information on the effects of salbutamol on human fertility. There were no adverse effects on fertility in animals.

Pregnancy
Administration of drugs during pregnancy should only be considered if the expected benefit to the mother is greater than any possible risk to the fetus.

During worldwide marketing experience, rare cases of various congenital anomalies, including cleft palate and limb defects have been reported in the offspring of patients being treated with VENTOLIN. Some of the mothers were taking multiple medications during their pregnancies. As no consistent pattern of defects can be discerned, and baseline rate for congenital anomalies is 2 to 3%, a relationship with salbutamol use cannot be established.

Lactation
As salbutamol is probably secreted in breast milk, its use in nursing mothers is not recommended unless the expected benefits outweigh any potential risk. It is not known whether salbutamol in breast milk has a harmful effect on the neonate.

Symptoms and Treatment of Overdose
The most common signs and symptoms of overdose with salbutamol are transient beta agonist pharmacologically mediated events.

Hypokalaemia may occur following overdosage with salbutamol. Serum potassium levels should be monitored.

Lactic acidosis has been reported in association with high therapeutic doses as well as overdoses of short-acting beta-agonist therapy, therefore monitoring for elevated serum lactate and consequent metabolic acidosis (particularly if there is persistence or worsening of tachypnea despite resolution of other signs of bronchospasm such as wheezing) may be indicated in the setting of overdose.

Recommended dose
CP Pharma Salbutamol CFC-Free Metered Dose Inhaler 100mcg/actuation has a duration of action of 4 to 6 hours in most patients.

Increasing use of beta-2 agonists may be a sign of worsening asthma. Under these conditions a reassessment of the patient's therapy plan may be required and concomitant glucocorticosteroid therapy should be considered.

As there may be adverse effects associated with excessive dosing, the dosage or frequency of administration should only be increased on medical advice.

CP Pharma Salbutamol CFC-Free Metered Dose Inhaler 100mcg/actuation is administered by the oral inhaled route only.

In patients who find co-ordination of a pressurised metered-dose inhaler difficult a spacer may be used with CP Pharma Salbutamol CFC-Free Metered Dose Inhaler 100mcg/actuation.

Babies and young children using the CP Pharma Salbutamol CFC-Free Metered Dose Inhaler 100mcg/actuation may benefit from the use of a paediatric spacer device with a face mask.

- RELIEF OF ACUTE BRONCHOSPASM**
- Adults
100 or 200 micrograms
 - Children
100 micrograms. The dose may be increased to 200 micrograms if required.

- PREVENTION OF ALLERGEN OR EXERCISE-INDUCED BRONCHOSPASM**
- Adults
200 micrograms before challenge or exertion.
 - Children
100 micrograms before challenge or exertion. The dose may be increased to 200 micrograms if required.

- CHRONIC THERAPY**
- Adults
Up to 200 micrograms 4 times daily
 - Children
Up to 200 micrograms 4 times daily.

On demand use of CP Pharma Salbutamol CFC-Free Metered Dose Inhaler 100mcg/actuation should not exceed four times daily. Reliance on such supplementary use of a sudden increase in dose indicates deteriorating asthma (see Warnings and Precautions).

INSTRUCTIONS OF USE / HANDLING:

A. Remove the cap. Should it be a new inhaler or that hasn't been used for several days, shake the inhaler and actuate it to ensure its good performance. Should the inhaler be periodically used read the following instructions:

B. Shake the inhaler.

C. Exhale as much air as possible from your lungs.

D. Adapt the inhaler to your mouth according to the position indicated in the illustration.

E. Breathe in as deeply as possible. You should press the apparatus, while you are breathing in.

F. Remove the inhaler from your mouth and try to hold the air in your lungs for a few seconds.

G. The oral actuator-adapter should be regularly washed. In order to do it, remove the actuator from the inhaler and wash it with abundant water.

H. Store it with the cap on in order to protect it from dust and dirt.

The bronchodilator effect of each application of inhaled salbutamol lasts for at least four hours, except in patients whose asthma is worsening. These patients should be warned not to increase the use of the inhaler but to consult the doctor in case an increase in the dose of the treatment with inhaled or systematically administered glucocorticosteroids is needed.

The need for an additional use or a sudden increase in the dose shows a worsening of the asthmatic disease. As adverse effects related to extremely high doses may occur, the dose or the administration frequency should only be increased on the doctor's orders.

Note: The information given here is limited. For further information consult your doctor or pharmacist.

Storage:
Store below 30°C. Protect from direct sunlight and do not freeze.

Pressurized inhaler:
It should not be perforated, broken or burnt even when it is apparently empty.

Presentation/Packing:
Aluminium aerosol can with 200 inhalations (10ml), containing 100µg of salbutamol each actuation, sealed with a metering valve (50µl of capacity), equipped with a polypropylene plastic oral actuator-adapter (or mouthpiece) and polypropylene plastic cap.

Route of Administration:
Respiratory (Inhalation)

Product registration holder:
HOVID Pharmacy Sdn. Bhd.,
121, Jalan Tunku Abdul Rahman,
30010 Ipoh, Perak, Malaysia.

Manufactured by:
LABORATORIO ALDO-UNION S.L.
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Barcelona, Spain.

Marketed by / Distributed by:
Caring Pharmacy Retail Management Sdn. Bhd. (757411-U)
No. 1, Jalan 51/203A, Kawasan Perindustrian Tiong Nam,
Seksyen 51, 46050 Petaling Jaya.

Date of Revision: Sept 2022